



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : زينب محمد عوده سليم

العنوان : 00.0.0.0.0

الحي / المدينة : الاخصاص

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 20-09-2026

رقم البطاقة : 25303032101221 رقم التليفون : 01202218199

العمر(تاريخ الميلاد) : 03-03-1953 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع:

المقرر بما فيه (.....) التوقيع:

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
 الاسم : د/ندى حسن الوظيفة : التوقيع :
 الاسم : د/نرمين رمضان الوظيفة : التوقيع :
 أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
 تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: زينب محمد عوده سليم National ID: 25303032101221	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوسناف
روستافي
9-14



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
سجل المصابين

09/13/2026

MDT FORM

Name:	Zainab Mohamoud Asudeh Slim	HRN:	7777
Age:	74 Y	ID:	25303032101221
Menopausal Sta:	postm	Phone:*	01202218199
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender:		Reason Of MDT:	
CO Morbidity:	cardiac , HTN	ECOG Performance Status	0 1 2 3 4
Surgical History:*	none		

Clinical Data:

BX: ILC
ER: 8/8 PR: 8/8 HER2: -ve KI67: 45 %

History:

postm rt breast cancer ct3 n2 mo ilc er8/8 pr8/8 her2-ve ki 45 %
cts rt hfl likely cyst for mri liver
mri liver done on 19/7/2026 bilobar hepatic cysts
cts also showed d12 and rt iliac lytic osseous lesions
bs free
clips inserted
pt received nact 3 ac 9 weeks taxol last 19/8/2026
Sonomammography:
new SM complete radiological response

Biomarker/Molecular

Mets Workup:

Recommendations:

for RT WLE (wire) + ALND as planned

MDT Outcome & Plan

RT WLE (wire) + ALND as planned



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

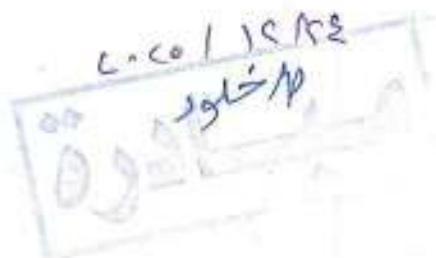
Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد كركاشي	د. محمد نور مكي		

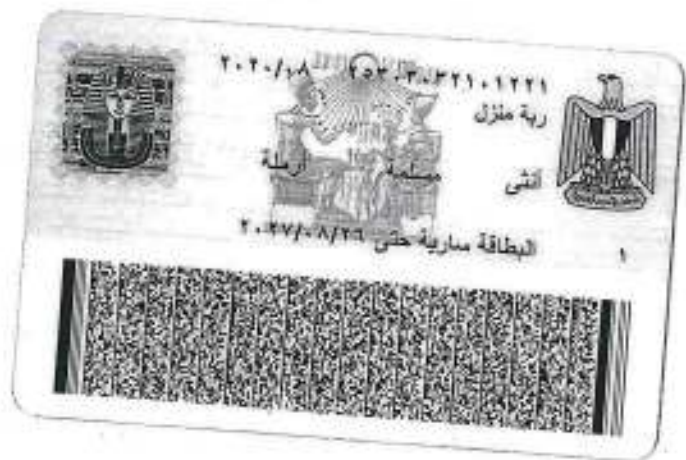


فتح ملف
دا امريال



٧٢
١١/١٢/٢٠٢٠
التي هي

٣٩/١٨
—————
٢٢٥



VVVV

ECHO REPORT

NAME: زينة محمد عود

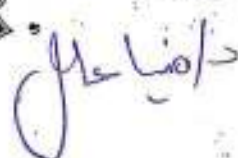
DATE: 14 / 4 / 2026

ID NUMBER: /

SEX: ♀ AGE: 72

EF	68.1	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD		2-4cm	LA	3.2	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

- LV Dimensions ———— oimel
- LV Wall Thickness ———— oimel
- LV Diastolic Function DD grade I
- SWMA ———— o SWMA
- LA and Ao Root Dimensions ———— oimel
- Mitral Valve ———— oimel
- Aortic Valve ———— oimel
- Tricuspid Valve ———— mild TR
- Pulmonary Valve ———— oimel
- Right Side and Systolic Function ———— oimel
- Intracardiac Masses and Effusion ———— o

DR. 

Patient ID: 92894	Patient Name English: Zainab Mohamed Oadah
Age: 72 Y	2026/02/01 10:06:12 AM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Right breast cancer (pathologically proven invasive lobular carcinoma, grade II)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Mild background enhancement.

ACR: C (heterogeneous dense)

Right breast:

- **Low energy (Non contrast) images:** Right breast shows middle third UOQ focal asymmetry, with internal suspicious microcalcifications.
- **Recombined (Post contrast) images:** right breast shows corresponding heterogeneous mass and non-mass enhancement, the whole area measures about 5.2x3.5 cm, seen at 7 cm from the nipple.
- **Complementary US:** Corresponding heterogeneously hypoechoic infiltrated parenchyma, mounting to mass formation.
 - Multiple enhancing pathological axillary LNs, corresponding by US to about 4 pathological infiltrated LNs, the largest measures 2.7x1.5 cm.
 - An indeterminate LN seen at level 2, with 0.36 mm focal cortical thickening.
 - A prominent 0.4 cm supraclavicular LN is noted.

Left breast:

- **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show No pathological mass or non-mass uptake. Two retroareolar tiny foci noted, corresponding to prominent ducts with small cysts by Complementary ultrasound.
- Normal skin / nipple / areolar complex.
 - No left enlarged axillary LNs.

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven ILC) UOQ mass and non-mass enhancement lesions as described above with ipsilateral pathological axillary LNs.
- **LEFT BREAST:** (BIRADS 2= benign looking) Retroareolar ductectasia along with adenosis.

Best regards,

Dr. Ghada Khaled, MSc

Patient ID: 94664	Patient Name English: Zainab Mohamed Oadah Salim
عمر ٧٢ سنة ١ شهر	2026/04/15 10:31:25 AM :Report Date

Ultrasound Guided clip insertion:

Clinical summary: Pathologically proven right breast invasive lobular carcinoma grade II (+ve ER, +ve PR, -ve HER2 and Ki67= 45 %) for clip insertion and bracketing.

Technique:

- Under complete sterile conditions, 3ml of local anaesthetic were administered at the site of concern of the right breast.
- By U/s the lesion seen at zone c measuring 28x10mm anterior to lesion there is alter tissue echogenicity with intraductal extension reaching nipple all in area about 70x30mm) .
- The metallic clip was inserted at most posterior margin of the malignant mass
- This was confirmed by mammography CC & ML views.

Best Regards
Dr. Marwa M.Hassan

VVVV

Patient Name : الأستاذ / زينب محمد عوده سليم	Request No. : 13003906
Patient No. : 25303032101221	Request Date : 10/12/2025
Path No. : 3828/25	Verification Date : 16/12/2025
Sex : Female	Financial Category : جومستاف رومني - مبادره
Age : 72 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- DCIS: Absent.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67%: 45%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 21/12/2025 12:59:07

Page 2 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / زينب محمد عوده سليم	Request No.	: 13003906
Patient No.	: 25303032101221	Request Date	: 10/12/2025
Path No.	: 3828/25	Verification Date	: 16/12/2025
Sex	: Female	Financial Category	: جوماتاف روسي - مبالغه
Age	: 72 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected on 10/12/2025, at 9:40 am.

Gross Examination:

- Three grey white tissue cores, the longest is 1.5 cm. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

* Histological type: Invasive classic lobular carcinoma.

* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).

* Ductal carcinoma in situ: Absent.

* Lobular carcinoma in situ: Florid.

* Angioinvasion: Absent.

* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).

- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).

- Her2: Negative membrane reaction in tumor cells (score 0).

- Ki67%: 45%.

Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive classic lobular carcinoma, Grade 2.

- Florid lobular carcinoma in situ.

Verified by :

Dr.Duaa Mohamed

Printing Date : 21/12/2025 12:59:07

Page 1 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Zainab Mohammed Odeh Selim
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 12-02-2026 11:06
Medical No. : 315736
Ins.Company : تعافد مستشفيات

- **Indication:** 72-year-old female with history of breast carcinoma. Evaluation for possible osseous metastatic disease.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection.

FINDINGS:

- No scintigraphic evidence of skeletal metastatic disease could be identified.
- Degenerative/Arthritic changes (remarkable, expected for age) are seen at different levels of the spine, sternum, both shoulders, as well as the weight-bearing joints.

IMPRESSION:

- No definite scintigraphic evidence of osseous metastases.
- Follow up is recommended.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Name : ZAINAB MOAHAMED OWDA

Branch : Portable

Ref.Physician Prof :

Date : 30-12-2025 01:10

Medical No. : GRI-25303032101221

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

No clinical data available.

I- CHEST:

- Right axillary pathologically enlarged lymph node measuring 25 x 20 mm in maximum dimensions.
- Few tiny bilateral scattered pulmonary nodules largest measuring 5 mm at the right lower lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. The right hepatic lobe shows a well defined segment VIII hypodense non enhancing focal lesion measuring 18 x 13 mm. No dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.

Name : ZAINAB MOAHAMED OWDA

Branch : Portable

Ref.Physician Prof :

Date : 30-12-2025 01:10

Medical No. : GRI-25303032101221

Ins.Company :

- No abdominal or pelvic lymphadenopathy.
- No ascites.
- D12 vertebral body and right iliac bone lytic areas are seen.

Opinion:

- ❖ Right axillary pathologically enlarged lymph node.
- ❖ Few tiny bilateral scattered pulmonary nodules.
- ❖ Right hepatic lobe segment VIII focal lesion ? cyst for dynamic MR study.
- ❖ D12 vertebral body and right iliac bone lytic areas for bone scan.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

	
وزارة الصحة Kingdom of Saudi Arabia	وزارة الصحة Kingdom of Saudi Arabia
9-14	09/13/2026

MDT FORM

Name:	Zainab Mohamoud Aoudeh Slim
Age:	74 y
Menopausal Sta	postm
NRS SCORE:	
BMI:	
Gender	
CO Morbidity:	cardiac , HTN
Surgical History:*	none
ECOG Performance Status	0 @ 10 20 30 4

MRN:	7777
ID:	25303032101221
Phone:*	01202218199
Marital Status:	
Family History:	
Reason OF MDT:	postneoadjuvant

Clinical Data:

BX:	ILC
ER:	8/8
PR:	8/8
HER2:	-ve
KI67:	45 %

History:

postm rt breast cancer ct3 n2 mo lhc er8/8 pr8/8 her2-ve ki 45 %

cts rt hñ likely cyst for mri liver

mri liver done on 19/7/2026 bilobar hepatic cysts

cts also showed d12 and rt iliac lytic osseous lesions

bs free

clips inserted

pt received nact 3 ac 9 weeks taxol last 19/8/2026

Sonography:

new SM complete radiological response

Biomarker/Molecular

Mets Workup:

Recommendations:

for RT WLE (wire) + ALND as planned

MDT Outcome & Plan

RT WLE (wire) + ALND as planned

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4
Right Side				Left Side			

Clinical Staging:

Pathological Staging:

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
------------------	---------	-----------	-----------

SIGNATURE